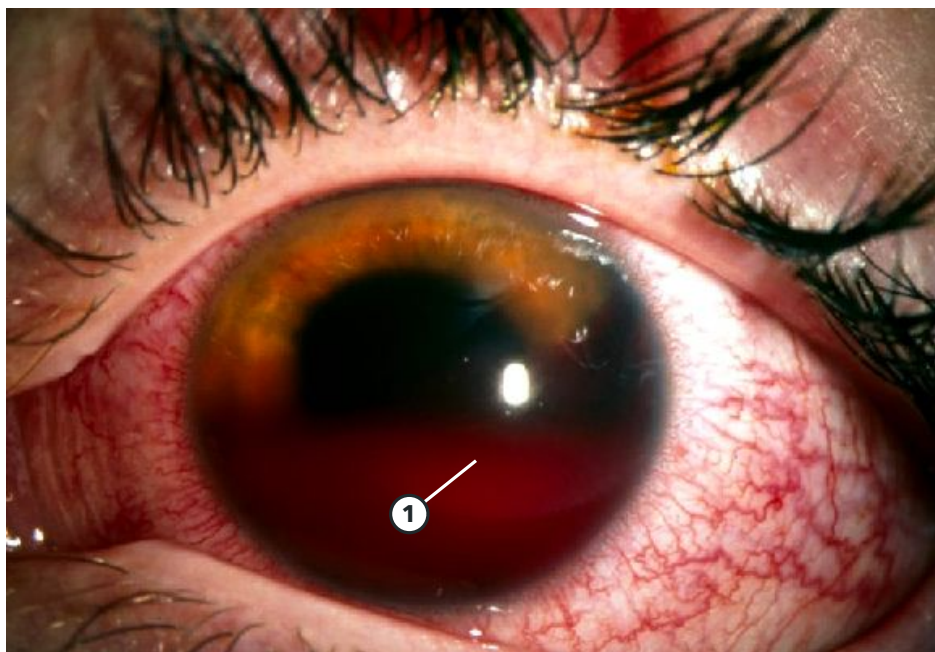


Hyphaema and pressure risk

CLINICAL M3-M5 | TYPE B | A focused decision guide

A small blood layer can carry high risk

Blood in the anterior chamber



1: layered blood below the pupil. This supplied photograph illustrates hyphaema, not its cause or severity in a particular case. Supplied M3 Anki, media 246.

Risk assessment after globe integrity is established

- 1 Record acuity, blood level, IOP and other traumatic injuries; ask about sickle-cell disease or trait.
- 2 Sickle haemoglobin increases the risk of outflow obstruction and optic-nerve injury even with relatively modest IOP.
- 3 Use specialist pressure management and a lower threshold for escalation; a small hyphaema is not sufficient reassurance.

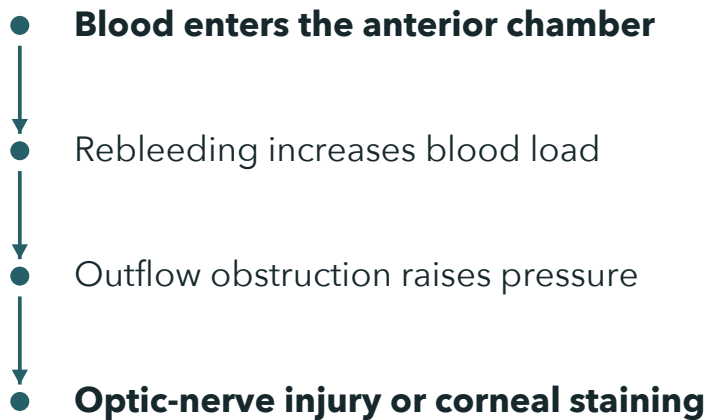
Medication matters

Carbonic anhydrase inhibitors require special caution in sickle haemoglobinopathy because acidosis can promote sickling; specialist regimens differ.

References AAO: Hyphaema | CDC: Sickle trait and post-hyphaema glaucoma

Prevent and detect secondary injury

The complication chain



Original mechanism schematic. A rebleed may increase blood load and pressure; not every patient develops every complication.

Protection and monitoring

Action	Reason
Shield, head elevation and restricted activity	Reduce further trauma and rebleeding risk.
Avoid nonessential aspirin or NSAIDs	Limit platelet-related bleeding risk; coordinate prescribed antithrombotics.
Scheduled IOP and corneal review	Detect pressure injury and corneal blood staining.

Escalation

Persistent pressure elevation, corneal blood staining or a worsening total hyphaema may require anterior-chamber washout. Decisions depend on duration, haemoglobinopathy and optic-nerve risk.